

THAT DEEPER FEELING

Conscious Intimacy Coaching

The Facilitator's Guide

& Practice Catalog

*For practitioners of the SPINE framework.
This document is not client-facing.*

Section 1: Who You Are In This Work

The Practitioner's Posture

You are not a therapist in this room — even if you are one outside it.

The distinction matters.

Therapy works primarily with what's broken. This framework works primarily with what's growing. The posture is different. The interventions are different. The relationship is different.

You are the person who holds the container. You create the conditions. You model that it's safe to be unsettled — that uncertainty is survivable, that desire is allowed, that the body can be trusted.

You are the evidence that leaving the system doesn't mean losing yourself.

The first qualification for facilitating this work is doing it.

The Practitioner's Own Map

Before you facilitate this work for anyone else, complete your own SPINE assessment. Not as homework. As honest inquiry.

Where is your anchor? What does your own shape look like? Where are your edges — the places where your own system-exit is still incomplete, where your own shame sources are still active?

You don't need to be finished with your own work to facilitate this. You need to be in it. The client needs to sense that you're not asking them to go somewhere you haven't been willing to go yourself.

Your map will also reveal your facilitation risks — the places where a client's material is most likely to activate your own. This isn't a problem. It's information. Supervised facilitators bring their own activation patterns to supervision regularly.

Reflection questions for your own map:

- Which domain activated most strongly when you completed the assessment?
- Where did your body check show distress or blankness?
- What shame sources still carry significant charge for you?
- Where is the gap between your current and vision scores widest?
- What is the one thing you're most afraid a client will bring to session?

Reading the Room

What flooding looks like:

Rapid speech, tears that won't stop, inability to track the conversation, repetitive circling over the same content, visible agitation. The nervous system is overwhelmed. Don't add content. Don't process. Ground. "Let's slow down. Feel your feet. Tell me one thing you can see right now." Stay with grounding until the nervous system settles. Then ask: "What just happened in your body?"

What shutdown looks like:

Flat affect, monosyllabic responses, gaze going vacant, "I don't know" to everything, body going still. The nervous system has withdrawn. Don't push through. Acknowledge: "Something just shifted. Your body went somewhere else." If they can't come back: "You don't need to come back right now. I'm here when you're ready." Sometimes the session's most important moment is the practitioner sitting quietly with someone who's gone offline.

What intellectualization looks like:

Articulate, analytical, insightful — and completely disconnected from the body. The client can describe their patterns perfectly while maintaining total distance from the felt experience. This is the system's most sophisticated defense — understanding as avoidance. Interrupt gently: "You're describing this really clearly. Where is it living in your body right now?"

What performance looks like:

The client who is "doing the work perfectly." Every practice completed, insightful observations, performative vulnerability. Watch for: the work being offered to you rather than experienced by them. "Who is this for right now? Is this for you, or for me?"

The Five Facilitation Principles

1. Curiosity over correction

You are not here to fix. You are here to wonder. When a client brings something — a shame pattern, a body response, a relational dynamic — the first move is always curiosity. “Tell me more about that.” Not: “Have you tried...” The moment you correct, you become the system.

2. Body before mind

The insight that matters is the one the body holds. When a client understands something intellectually but the body hasn't caught up: the body is right. Always check in with the body before, during, and after any significant work. “Where does that land right now?”

3. Both irreducible moments, always

Every significant shift in this framework involves two experiences: the unauthorized feeling (something true that wasn't allowed inside the system) and the non-evaluative witness (someone who receives it without judgment). Your primary job is the second one. Create the conditions for the first and then be the witness.

4. The session belongs to them

Your agenda is secondary. If a client arrives with something alive and urgent, follow it — even if it means the planned work doesn't happen. The practice prescription is a guide, not a curriculum. What's alive in the room is always more important than what's planned on the page.

5. Know your edges

You will encounter material that activates you. Orientation. Desire. Trauma. Theology. Know where your edges are. Know what you need to bring to supervision. The client doesn't need you to be unactivated. They need you to be honest about your activation rather than performing neutrality.

Section 2: The Differential Guide

Every sub-dimension on the SPINE map falls into one of three categories based on the intersection of score, shame intensity, and body response. The category determines what kind of intervention is appropriate — and what kind will do harm.

Resourcing

Score under 50%, low shame, body at curiosity or mild resistance. This territory wasn't heavily damaged — it just wasn't built. The client needs new input, new experiences, new vocabulary.

What it looks like in the room:

Curiosity. Openness. Energy. The client leans in when you introduce practices in this domain. They may even enjoy it. That enjoyment is real — this is the growth edge, not the wound edge.

What helps:

Exploration. New language. Permission. The Pleasure Minute, The Want Before the Why, The Curiosity Shelf. Practices that introduce something new rather than processing something old.

What doesn't:

Deep somatic processing. Intensive shame work. Trauma protocols. The territory doesn't need healing — it needs building. Treating it like a wound creates a wound where there wasn't one.

When to refer:

When the apparent low shame turns out to be dissociation. If the body check is blankness rather than curiosity, re-assess: what looks like readiness may be disconnection.

Pattern

Score under 60%, moderate shame (3–4), body at resistance or significant resistance. The old system's patterns are still running — not as acute wounds but as default operating procedures. The client needs to see them, name them, and interrupt them.

What it looks like in the room:

Resistance that has intelligence in it. The client isn't flooding — they're bracing. They may argue, intellectualize, or deflect. The resistance is the pattern in real time. Name it, gently.

What helps:

Naming. Visibility. Choice points. The Watchtower, The Mirror Voice, The Deed, The Body No. Practices that interrupt the automatic sequence and insert a moment of choice.

What doesn't:

Pushing through the resistance. Behavioral prescriptions that bypass the body. "Just try saying no more." The pattern needs to be seen and understood before it can be changed.

When to refer:

When pattern work reveals deeper trauma underneath the patterns. When the resistance escalates rather than softening over multiple sessions. When the patterns are relational and require couples or family therapy to address.

Healing

Score under 50%, high shame (4–5), body at distress or blankness. This territory carries active wounding. The nervous system is in protection mode. Intervention must be slow, titrated, and trauma-informed.

What it looks like in the room:

Distress. Flooding. Shutdown. Tears that come before words. Body responses that are disproportionate to the content. Dissociation — the client leaves the room without physically leaving. These are not problems. They are the body saying: this territory is not safe yet.

What helps:

Titration. Slow, repeated, body-first practices. The Daily Return, The Lighthouse, The Witnessed Voice. Grounding before and after every session that touches this territory. Concurrent therapy for anything that activates traumatic memory.

What doesn't:

Moving fast. Cognitive reframing. "You're safe now" (the body decides when it's safe, not the practitioner). Exposure without titration. Processing without grounding. Insight without integration.

When to refer:

Always refer for concurrent therapy when a domain shows healing indicators. This work is not therapy. When stability is below 5/10, when trauma responses are acute and persistent, when suicidal ideation or self-harm is present — these require clinical containment that this framework is not designed to provide.

Section 3: The Complete Practice Catalog

Every practice in this catalog is written twice: once in Kimberly's voice for the client, and once in clinical language for the practitioner. The client version is what gets shared. The practitioner version is what informs your facilitation.

THE DAILY RETURN

Embodiment · Healing · Weeks 1–4

FOR THE CLIENT:

Before the day gets its hands on you — just after waking, while your body is still warm and unguarded — find your own hands. Let them land somewhere without deciding first. Belly. Heart. Thigh. Ribs. Wherever they go is already information.

Stay there. Not to fix anything. Not to feel anything in particular. Just to notice you're here. That this body — this specific, imperfect, lived-in body — is yours. It has always been yours. Nobody ever had the right to make you a stranger inside it.

Sixty seconds. One hand. One breath that's a little longer going out than coming in. And then, quietly, to yourself: I live here.

Do it again before sleep. The body learns through repetition what words alone can't teach.

FOR THE PRACTITIONER:

What this practice is actually doing:

Creating repeated, uncharged somatic contact with the body. The mechanism is simple — the nervous system learns what gets repeated. Repeated inhabitation without consequence gradually updates the threat assessment the body has around being present in itself.

How to introduce it in session:

"I want to give you one practice before our next session — just one, nothing else. It takes sixty seconds. That's not metaphor — literally sixty seconds, twice a day. The only instruction is to find your own hands and let them land somewhere on your body, and stay for one breath." Pause. "Can you do that?" Getting a verbal yes is part of the practice — it's the first act of self-authorization.

What to watch for:

The client who reports numbness — this is the practice working, not failing. The nervous system protects through disconnection. Numbness means they're present enough to notice disconnection. The client who floods — slow down, add more grounding before the body contact, add the shame interrupt as a companion practice. The client who skips it consistently — that avoidance is the most important clinical data. Bring it to session. "What happens when you think about doing it?" Not judgment — genuine curiosity.

When to adapt it:

For clients with significant body shame or history of physical violation, start with hands on the clothed thighs or on the surface of a chair — anywhere that feels less charged than belly or chest. The location matters less than the contact.

When to pause it:

If the practice consistently produces acute distress that doesn't resolve within two to three minutes — pause and bring a somatic therapist in before continuing.

Modality roots:

Somatic experiencing (Peter Levine), sensorimotor psychotherapy (Pat Ogden), polyvagal theory application (Deb Dana).

THE LIGHTHOUSE

Nourishment • Healing • Weeks 3–8

FOR THE CLIENT:

Once a day — it takes two minutes — find one moment of genuine pleasure from the last twenty-four hours. Something small is perfect. The first sip of something warm. A song that moved through you. Sunlight on skin. The specific satisfaction of a door closing behind you at the end of the day.

Write it down. Just the moment and what it felt like in your body. Not an analysis. Not gratitude. Just: this happened, and my body registered it as good.

Then close your eyes and let your body re-experience it for thirty seconds. Let the pleasure be as large as it wants to be.

This is not a small practice. You were taught that pleasure was dangerous. You are methodically, tenderly, teaching your nervous system something different: pleasure is safe data. I am allowed to feel good.

The lighthouse doesn't move toward the storm. It stands and keeps its light on. That's what this practice is — keeping the light on for the parts of you that have been navigating in the dark.

FOR THE PRACTITIONER:

What this practice is actually doing:

Systematic pleasure sensitization. The system created a near-automatic shame interrupt that fires when pleasure registers — arousal → shame, enjoyment → guilt, receiving → unworthiness. This practice builds a new pathway: pleasure → notice → stay → record → re-experience. Each repetition is a small update to the nervous system's threat map.

How to introduce it in session:

"Before we look at the bigger landscape — can we start with something specific? What was one moment in the last week that your body registered as genuinely good? Even tiny. Even something you almost didn't notice." Let them find one. Then: "That's the practice. Once a day, find one moment like that and write it down. That's all. We're not trying to transform your relationship to pleasure in a week. We're just keeping the light on."

What to watch for:

Clients who cannot find a single moment of pleasure — this is significant clinical data about the depth of the shutdown. Work at even smaller granularity: not pleasure but simply not-pain. A moment that was neutral or mildly okay. Build from there. Clients who find the practice and then immediately contextualize it — “I enjoyed the coffee but then I felt guilty about spending the money.” The guilt-following-pleasure pattern is exactly what we’re tracking. Name it, gently, as information: “The pleasure arrived. And then something else arrived right behind it.”

When to adapt it:

For clients with acute shame around pleasure, start with non-sensory pleasure — an intellectual satisfaction, a moment of competence, a good conversation. Build toward physical/sensory pleasure gradually.

Modality roots:

Mindfulness-based sex therapy (Lori Brotto), sensate focus (Masters and Johnson, adapted), positive neuroplasticity training (Rick Hanson’s “taking in the good”).

THE WITNESSED VOICE

Power • Healing • Weeks 4–8

FOR THE CLIENT:

Find a private minute. Hit record on your phone. Then say something true — something that’s been living inside you without sound.

It doesn’t have to be profound. I’m tired of performing. I want more than this. I don’t actually know what I want yet and that scares me. Whatever is true right now.

You don’t have to listen back. You don’t have to share it with anyone. The practice is simply this: your voice, speaking the true thing, into the air, where it becomes real.

The system taught you that your voice — especially in dissent, in desire, in need — was dangerous. This practice disagrees. Quietly. Privately. One true sentence at a time.

There's something that happens when a true thing gets spoken aloud rather than staying in the body as thought. It becomes real in a different way. It takes up space. It arrives in the world as yours.

That's the whole practice. Say the true thing. Let it be real.

FOR THE PRACTITIONER:

What this practice is actually doing:

Externalizing internal experience through voice — one of the most powerful mechanisms for integrating what has been held in isolation. The specific instruction not to listen back removes the evaluative loop (I said it → I judge how it sounded → I manage the next saying). The voice goes out without being reviewed. Over time this builds the trust between knowing and saying.

How to introduce it in session:

"There's a practice I want to offer you, and it's completely private — you don't share it with me, you don't even have to listen to it yourself. You just say one true thing into your phone and let it go into the world." Watch their response. If there's immediate resistance: "What's coming up as I describe that?" The resistance is the practice's first gift.

What to watch for:

The client who makes the recordings but fills them with qualified, careful, palatable truths. The practice should gradually move toward rawer material. If it doesn't, gently ask: "What's the thing you haven't recorded yet?" The client who cries during the first recording — this is profound release, not dysfunction. Hold it with care.

Modality roots:

Narrative therapy, expressive arts therapy, voice and somatic work (Nadine George Voice Work).

THE WATCHTOWER

Sovereignty • Pattern • Weeks 5–8

FOR THE CLIENT:

A few times a day — especially when you notice yourself monitoring, reviewing, scanning for approval — ask one question:

Am I in the tower?

The tower is where you go when the old surveillance program is running. You know the feeling: checking how you're coming across, rehearsing explanations, bracing for consequences that aren't actually coming. Monitoring yourself from above, through someone else's eyes.

When you notice you're there: name it. I'm in the tower. No judgment. Just accurate location. Then find the ground. Feet on the floor. One thing you can touch right now. One breath out that's longer than the one before it.

That's the climb down. You don't have to stay at ground level forever. Just practice knowing it exists.

FOR THE PRACTITIONER:

What this practice is actually doing:

Interrupting the hypervigilance cycle that was adaptive inside the system (constant monitoring kept people safe from punishment) and is now running as default state outside it. The naming practice ("I'm in the tower") activates the prefrontal cortex's observing function — you cannot name an experience without having a degree of separation from it. That separation is the beginning of choice.

How to introduce it in session:

Tell the story first. "The system trained your nervous system to stay in a watchtower — permanently scanning for evaluation, for approval, for the consequence that might be coming. That was genuinely useful then. It's what kept you safe. It's running now without the threats it was built for." Then: "The only practice is noticing. Am I in the tower right now?" Ask them in session: "Are you in the tower right now?" Often the answer is yes, and that in-session noticing is more powerful than any homework.

What to watch for:

The client who can name the tower state intellectually but doesn't feel the descent. The naming has become another performance. Bring it to the body: "When you're in the tower — where do you feel that physically?" Then: "What shifts, even slightly, when you feel your feet on the floor right now?"

Modality roots:

Polyvagal theory (Stephen Porges, Deb Dana), IFS (identifying the protector parts), mindfulness-based approaches.

THE MIRROR VOICE

Power • Pattern • Weeks 3–8

FOR THE CLIENT:

Stand in front of a mirror. Make real eye contact with yourself — not a glance, actual contact. Then say one true sentence out loud.

Start wherever you are. I'm here. I'm tired. I want something I haven't let myself want yet. The words matter less than the contact — you, meeting yourself, in the mirror, with your actual voice, saying something actually true.

Three times a week. Two minutes. Build from small truths to larger ones across the weeks.

By week four, let yourself say something about what you need. By week six, something about desire. By week eight, something you've wanted in your intimate life that you've been keeping in the quiet.

FOR THE PRACTITIONER:

What this practice is actually doing:

Building the self-as-witness capacity. The system removed the client as reliable witness to their own experience — every self-perception was filtered through the system's evaluation. The mirror practice re-installs the self as a legitimate, trustworthy witness. The progressive build from neutral to intimate content gradually expands what can be witnessed.

How to introduce it in session:

"How do you feel about mirrors generally?" The answer is clinical data. Many people from these systems have a fraught relationship with mirrors — the body under evaluation. Start there if needed. "What if the mirror was just for contact —

not evaluation?" Introduce the practice as experiment, not prescription.

What to watch for:

Inability to maintain eye contact — very common, important, not a problem. "Notice what happens when you make eye contact with yourself. What's the first thing that comes?" Laughter is often the first thing — the nervous system releasing through humor. Tears are also common. Blankness — the same disconnection present in other domains, worth noting.

Modality roots:

Person-centered therapy (unconditional positive regard, self-as-subject), psychodrama, mindful self-compassion (Kristin Neff).

THE DEED

Power • Pattern • Weeks 5–8

FOR THE CLIENT:

Once a week, find something you want to do — small enough to complete in a day — that you've been waiting for permission to do. Not dramatic. Not a declaration. Just something small that's genuinely yours.

Take it. Do it. Then write three words: I chose this.

Week five: claim one hour of time that's purely, specifically yours. Week six: hold one feeling all the way through without routing it through anyone's evaluation. Week seven: say one true thing you'd normally keep to yourself. Week eight: acknowledge one desire — let it exist without acting on it or suppressing it.

Each week, the territory gets a little more interior. That's intentional. You're not just practicing doing things. You're practicing being the one who decides.

FOR THE PRACTITIONER:

What this practice is actually doing:

Graduated self-authorization practice. The progressive structure is essential — starting with time (external, behavioral) and moving toward desire (internal, relational) mirrors the natural sequence of reclaiming power. The written “I chose this” creates a somatic anchor for the felt sense of genuine choice — over many repetitions, the body begins to recognize genuine choosing as distinct from compliance.

How to introduce it in session:

“What’s one thing you’ve wanted to do this week that you haven’t given yourself permission for?” Whatever they name — check: is it genuinely theirs, or is it reactive to the system? If it’s reactive: “Is that something you actually want, or something you’ve decided you should want because the system said you shouldn’t?” Real want vs. pendulum swing. Both are worth exploring, but they’re different practices.

What to watch for:

The client who chooses things that are still fundamentally organized around pleasing or performing — “I gave myself permission to rest, but I made sure the house was clean first.” The permission is real but partial. Name it gently: “I notice you still had to earn it.” Also: the client who can’t think of anything they want. That blankness about wanting is one of the most important things to sit with.

Modality roots:

ACT (values clarification and committed action), behavioral activation, somatic experiencing.

THE BODY NO

Power • Pattern • Weeks 6–8

FOR THE CLIENT:

Your no lives in your body before it becomes a word. It might be a tightening in your chest. Your stomach pulling back. A subtle closing. A breath that holds.

This practice has one instruction: find your no before you need to use it.

In a low-stakes moment — someone offers you something you don’t want, asks for something that doesn’t feel right — don’t say the no yet. Just notice the physical signal first. Name it internally, wherever it lives:

there it is.

Your no is not an obstacle to intimacy. It is the architecture of it. No one can trust a yes that doesn't have a no available. Including you.

FOR THE PRACTITIONER:

What this practice is actually doing:

Rebuilding the somatic-to-verbal pathway for refusal. The system specifically damaged this pathway — saying no was spiritually, relationally, and sometimes physically dangerous. The body learned to suppress the no signal before it became conscious. This practice works at the level of signal recovery, not behavioral change — finding the signal first, before asking anything of it.

How to introduce it in session:

"I want to ask you something. In your body, right now — do you know where your no lives? When you imagine declining something you don't want, where do you feel that?" If they can locate it: "That's what we're going to pay attention to this week." If they can't: start more broadly with any body sensation connected to preference — yes and no begin as the same somatic vocabulary.

What to watch for:

For clients in relationships where refusal has been dangerous, the in-session practice of finding the no is sufficient for weeks six and seven. Do not rush to actual external refusal. The body knowing its own no is the entire first phase of this work.

When to adapt it:

For clients in relationships where refusal has been dangerous, the in-session practice of finding the no is sufficient for weeks six and seven. Do not rush to actual external refusal.

Modality roots:

Wheel of Consent (Betty Martin), somatic experiencing, trauma-informed sex therapy.



THE PLEASURE MINUTE

Nourishment • Resourcing • Weeks 3–6

FOR THE CLIENT:

One minute. That's it.

Find something that is purely, simply pleasurable to one of your senses. Not productive. Not meaningful. Not earned. Just: this tastes good, or this feels good on my skin, or this sound is beautiful to me.

Set a timer for one minute. Stay with the pleasure. The only instruction is to stay — not to analyze it, not to catalog it, not to appreciate it. Just to be in it.

When guilt arrives — and it may — let it stand at the door. It doesn't have to come in. The pleasure gets the minute. The guilt can wait.

You're allowed to feel good. Not because you've earned it. Just because you're here.

FOR THE PRACTITIONER:

What this practice is actually doing:

Expanding the window of pleasure tolerance. Most people from these systems have a very narrow pleasure window — they can tolerate brief, earned, justified pleasure. This practice gradually widens that window through repeated small experiences of staying with pleasure past the point where the shame interrupt usually fires.

How to introduce it in session:

Make it as low-stakes as possible. "Something with a good taste or a good texture or a good sound — something genuinely sensory, not an achievement." The client who says "I enjoy reading" — that's cognitive pleasure. We want somatic first. Help them find something physical.

Modality roots:

Sensate focus (adapted), mindfulness-based sex therapy, somatic experiencing.



THE ORIGIN QUESTION

Sovereignty · Resourcing · Any week

FOR THE CLIENT:

When shame arrives — and it will, because it does — ask one question before you accept it:

Where did I first learn this was shameful?

Not as a way to avoid the shame. Not to intellectualize your way out of it. Just to check: is this shame generated, or is this shame installed?

Generated shame says: I actually did something that conflicts with my own values. That shame is useful — it's information about who I want to be.

Installed shame says: someone told me this was shameful, and I absorbed the verdict before I had the capacity to question it.

When the answer traces back to a doctrine, a parent's voice, a community rule, a system — that's installed shame. And installed shame has a return address.

FOR THE PRACTITIONER:

What this practice is actually doing:

Installing a discriminatory capacity between authentic moral response and installed cultural/religious shame. This is foundational psychoeducation but delivered as a somatic practice — the client is invited to feel the difference between the two kinds of shame, not just understand it conceptually.

How to introduce it in session:

Often best introduced in session when shame is present live. "Can we stay with that for a moment? When that shame arrived just now — do you know where it came from? Not where you first felt it — where it was originally taught." The in-session version is more powerful than any homework.

Modality roots:

Narrative therapy (externalizing), IFS (identifying the origin of critical/shaming parts), feminist psychology.

THE WANT BEFORE THE WHY

Nourishment • Resourcing • Weeks 2–6

FOR THE CLIENT:

When a want arises — any want — practice saying it to yourself before evaluating it.

I want ____.

Nothing after the sentence yet. Not: I want this but I probably shouldn't. Not: I want this but it's too much. Just: I want ____.

Let the want exist in the room for ten seconds before you do anything with it. Ten seconds of the want being real before the evaluation begins.

Over time the ten seconds becomes twenty. Then a minute. Then you find yourself noticing what you want with a kind of spacious curiosity rather than a running moral audit.

That spaciousness is your appetite coming back to life.

THE SOMATIC CHECK-IN

All • Resourcing • Every session

FOR THE CLIENT:

Before any significant conversation, any intimate encounter, any moment that matters — thirty seconds.

Hand on belly. Hand on heart. One breath that goes all the way down.

Then ask: What's here?

Not what should be here. Not what you want to be here. What's actually present in your body right now.

This practice does one thing: it brings you into the room before the room begins. It makes you a participant rather than someone things happen to.

Thirty seconds. One hand. One question. That's enough to change what follows.

THE RETURN LETTER

Sovereignty • Pattern • Single session

FOR THE CLIENT:

Choose one shame. One specific thing you carry that the system gave you — desire, doubt, orientation, the wanting itself, the body you live in, the questions you couldn't stop asking.

Write it a letter. Three sentences is enough:

This shame was handed to me. I did not choose it and I did not generate it. I return it to its source, and I reclaim the space it occupied.

Read it out loud. Then do something ritual with it — burn it, bury it, tear it into small pieces and let them go. The ritual matters because the body learns through ceremony what the mind understands through information.

The shame may return — installed things don't leave cleanly. But when it returns, something will be different. There's a returned letter in your history now. You've said, on record, in your own voice: this was never mine to carry.

THE LABORATORY

Identity / Nourishment • Exploring • Weeks 4 onward

FOR THE CLIENT:

Something drew your attention this week. An attraction, a curiosity, a pull toward something you've either never let yourself look at directly or something that surprised you.

Write it down without evaluation. Then three questions:

What drew me? What did I notice in my body when I paid attention? Would I be curious to look at this again?

No conclusions. No decisions. No meaning-making yet. Just data collection.

You are a hypothesis worth testing. The lab is open. What are you curious about today?

THE ARRIVAL PRACTICE

Embodiment • Pattern • Before intimate encounters

FOR THE CLIENT:

Before intimacy — with yourself or another person — three minutes of arriving first.

Alone, before anything else begins: both hands on your body. One breath to feel your feet on the floor. One question, asked with genuine curiosity rather than evaluation: what is alive in my body right now?

Not: am I ready? Not: should I want this? Just: what's actually present?

This practice creates a tiny, important pause between stimulus and response — a breath of space where you exist as a subject, not an object. In that space, you can choose. From the inside.

That pause is sovereignty in practice.

Section 4: The Session Guide

Standard Session Arc

Opening (5–10 minutes)

Body check-in. One word for the body, one word for the mind. No commentary from the practitioner — just receiving. Then: “What are you bringing today?”

Practice Review (5–10 minutes)

“What did you notice?” Not “did you do it” — “what did you notice?” The practice is a data-collection mechanism. The data is what matters.

If they didn’t do it: “What happened when you thought about doing it?” Avoidance is always information. Never shame.

Session Work (30–40 minutes)

One axis or theme from their results map. Body-first throughout. When in doubt — “where does that land in your body right now?”

Closing (10 minutes)

“What’s landing from today?” One practice for the week — specific, achievable, connected to today’s work. Grounding. “Where are you leaving from today — in your body?”